Active Home Healthcare LLC

OASIS

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Las Vegas NV 89146
Phone: (775)471-9815 Fax: (702)726-9551
NPI: 1548015142

STAFF INFO
Name: Dizon, Bright RN
Patient INFO
Name: BRYANT, ROBERT
DOB: 1955-01-05

OASIS Assessment: OASIS E2 - START OF CARE

ADMINISTRATIVE INFORMATION

(M0010) Agency Medicare Provider Number:
297370

(M0014) Branch State:
Branch State: **Nevada**

(M0016) Branch ID Number:

(M0018) Attending Physician (NPI):
1023776309

Physician:
BAUTISTA, JAYSON

Physician Address:
138 SERPENS AVE LAS VEGAS NV 89183

☐ Not Applicable

(M0020) Patient ID number:
116336

(M0030) Start of Care Date:
04/16/2026

(M0032) Resumption of Care Date:
mm/dd/yyyy

☑ Not Applicable

(M0040) Patient Name:

First Name:	Middle Name:	Last Name:	Suffix:
ROBERT		BRYANT	

(M0066) Birth Date:
01/05/1955

(A0810) Sex:
◉ 1. Male ○ 2. Female

(M0050) Patient State of Residence:
State of Residence: **Nevada**

(M0060) Patient Zip Code:
89107

(M0063) Medicare Number:
8EV2N09WY84

☐ Not Applicable

(M0064) Social Security Number:

☑ Not Applicable

(M0065) Medicaid Number:

☑ Not Applicable

(M0080) Discipline Completing Assessment:
◉ 1. RN ○ 2. PT ○ 3. SLP/ST ○ 4. OT

Assigned Staff:
Dizon, Bright RN

(M0090) Date Assessment Completed:
04/16/2026

(M0100) Assessment Reason:
◉ 1. Start of Care

(A1005) Ethnicity Are you of Hispanic, Latino/a, or Spanish origin? Check all that apply:

- ☑ A. No, not of Hispanic, Latino/a, or Spanish origin
- ☐ B. Yes, Mexican, Mexican American, Chicano/a
- ☐ C. Yes, Puerto Rican
- ☐ D. Yes, Cuban
- ☐ E. Yes, Another Hispanic, Latino, or Spanish origin
- ☐ X. Patient unable to respond
- ☐ Y. Patient declines to respond

(A1010) Race What is your race?

- ☐ A. White
- ☑ B. Black or African American
- ☐ C. American Indian or Alaska Native
- ☐ D. Asian Indian
- ☐ E. Chinese
- ☐ F. Filipino
- ☐ G. Japanese
- ☐ H. Korean
- ☐ I. Vietnamese
- ☐ J. Other Asian
- ☐ K. Native Hawaiian
- ☐ L. Guamanian or Chamorro
- ☐ M. Samoan
- ☐ N. Other Pacific Islander
- ☐ X. Patient unable to respond
- ☐ Y. Patient declines to respond
- ☐ Z. None of the above

(A1110) Language:

A. What is your preferred language?
English

B. Do you need or want an interpreter to communicate with a doctor or health care staff?
◉ 0. No ○ 1.Yes ○ 9. Unable to determine

(B1300) Health Literacy:
How often do you need to have someone help you when you read instructions, pamphlets, or other written material from your doctor or pharmacy?
◉ 0. Never ○ 1. Rarely ○ 2. Sometimes ○ 3. Often ○ 4. Always ○ 7. Patient declines to respond ○ 8. Patient unable to respond

(M0102) Date of Physician-ordered Start of Care (Resumption of Care):
If the physician indicated a specific start of care (resumption of care) date when the patient was referred for home health services, record the date specified.
Skip to A1255, Transportation, if date entered:
04/16/2026

☐ NA - No specific SOC/ROC date ordered by physician

(M0104) Date of Referral:
Indicate the date that the written or verbal referral for initiation or resumption of care was received by the HHA:
mm/dd/yyyy

(M0150) Current Payment Sources for Home Care:
(Mark all that apply)

- ☐ 0. None; no charge for current services
- ☑ 1. Medicare (Traditional fee-for-service)
- ☐ 2. Medicare (HMO/managed care/Advantage plan)
- ☐ 3. Medicaid (Traditional fee-for-service)
- ☐ 6. Title programs (for example, Title III, V, or XX)
- ☐ 7. Other government (for example, TriCare, VA)
- ☐ 8. Private insurance
- ☐ 9. Private HMO/managed care

- [ ] 4. Medicaid (HMO/managed care)
- [ ] 5. Worker's compensation
- [ ] 10. Self-pay
- [ ] 11. Other (Specify):
- [ ] UK - Unknown

(A1255) Transportation (NACHC©):

In the past 12 months, has lack of reliable transportation kept you from medical appointments, meetings, work or from getting things needed for daily living?

- (●) 0. Yes
- ( ) 1. No
- ( ) 7. Patient declines to respond
- ( ) 8. Patient unable to respond

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(M1000) Inpatient Facilities that patient was discharged within the past 14 days?

(Mark all that apply)

- [ ] 1. Long-term nursing facility (NF)
- [ ] 2. Skilled nursing facility (SNF/TCU)
- [ ] 3. Short-stay acute hospital (IPPS)
- [ ] 4. Long-term care hospital (LTCH)
- [ ] 5. Inpatient rehabilitation hospital or unit (IRF)
- [ ] 6. Psychiatric hospital or unit
- [ ] 7. Other (specify):
- [x] NA - Patient was not discharged from an inpatient facility

(M1005) Inpatient Discharge Date (Most Recent):

mm/dd/yyyy [ ] UK - Unknown

Primary Reason for Home Health/Medical Necessity (POC Element):

Robert Bryant, a 71-year-old male patient admitted to home health care because they have significant functional impairments and medical conditions that severely limit their ability to leave the home without assistance. The patient is considered homebound as the patient ambulates with a hemi-cane and uses a motorized wheelchair for long distances due to right-sided hemiparesis following a cerebrovascular accident (CVA), which causes generalized muscle weakness, poor endurance, unsteady gait, and poor balance. The patient fatigues easily and experiences dyspnea with moderate exertion, making any attempt to leave the home a considerable and taxing effort. Additionally, the patient requires assistance with all activities of daily living, including grooming, bathing, dressing, transferring, toileting, and managing medications.

The patient's home environment also presents safety concerns, such as the presence of rugs in hallways, lack of grab bars in the bathroom, and poor lighting, which increase the risk of falls and injury if the patient attempts to leave the home independently. The patient has a history of urinary incontinence and contractures affecting the right arm and hand, further contributing to functional limitations. Allowable absences from the home are limited to medically necessary appointments, as outpatient therapy is currently deferred pending evaluation of palpitations related to medication.

Given these limitations and safety concerns, skilled nursing services are medically necessary to monitor vital signs, provide education on disease processes and medication management, assess for progression of disease, and reduce the risk of hospitalization and injury. Occupational therapy is also indicated to improve upper body strength, conditioning, and functional status to maximize the patient's independence and safety at home. These documented functional deficits and medical conditions clearly meet Medicare criteria for homebound status and justify the need for skilled home health services.

Homebound Reason (POC Element):

- [ ] Document as Clinical Narrative
- [ ] Patient is not Homebound

Criteria One (One must be met): *Describe what assistive devices the patient is dependent on (if any) and specifically why the patient condition warrants the special transportation or the assistance of another person or describe the condition that makes leaving home medically contraindicated.*

Criteria 1 A: Because of illness or injury

- [x] Patient needs the aid of supportive devices
  - [ ] Crutches
  - [x] Cane
  - [ ] Manual Wheel Chair
  - [ ] Walker
  - [x] Motorized Wheelchair
  - [ ] Scooter
- [ ] Patient Needs the use of special transportation.
- [x] Patient needs the assistance of another person to leave the place of residence

Criteria 1 B: Has a condition such as leaving his or her home is medically contraindicated.

- [ ] Status post surgery
- [ ] Angina upon minimal exertion
- [ ] Immuno deficiency

Other Homebound Reason :

Criteria Two (Both must be met)

Criteria 2 A: There must be exist a normal inability to leave home.

- [ ] Dyspnea on minimal exertion
- [ ] Bedbound
- [ ] Chair bound
- [ ] Dementia
- [ ] Blindness
- [ ] Angina upon minimal exertion
- [x] Paralysis
  - [ ] Quadriplegic or
  - [ ] Paraplegic or
  - [x] Hemiplegia
- [x] Unable to tolerate standing or walking for extended periods
- [x] Experiencing pain impairs/restricts mobility
- [x] Balance impaired with potential for falls; uses walls and furniture to ambulate
- [x] Leaving home is exhausting, extended rest periods to recover
- [x] Extended respiratory recovery after exertion
- [x] Requires human assist due to unsteady gait/poor balance
- [ ] Weakness and pain from recent surgery
- [x] Visually impaired-requires constant attendance/assist
- [ ] Cognitively impaired, unable to safely leave home without assist
- [ ] Infectious disease process causing weakness and exhaustion
- [ ] Narcotics impaired ability to safely leave home unassisted
- [ ] Bedbound and requires EMS support

Other Homebound Reason :

Criteria 2 B: Leaving home must require a considerable and taxing effort

- [ ] Special transportation
- [x] Severe pain on ambulation
- [ ] Leaving familiar environment causes fear and confusion
- [x] Leaving home is strenous requiring extended rest to recover
- [ ] Fatigue to arrange/use appropriate transportation
- [x] Requires assistive device to ambulate
- [ ] Exhausting to navigate vehicle and require human assist
- [ ] Requires assist to manage equipment and public transportation
- [x] Needs assistance to handle and transport equipment
- [x] Activities restricted due to pain limitations

Other Homebound Reason :

Prognosis (POC Element (§484.60 (2.v))):

○ 1-Poor ○ 2-Guarded ● 3-Fair ○ 4-Good ○ 5-Excellent

Functional Limitations (POC Element (§484.60 (2.vi))):

- ○ 1 - Amputation
- ● 2 - Bowel/Bladder (Incontinence)
- ● 3 - Contracture
- ● B- Other (Specify)
- ○ 4 - Hearing
- ● 5 - Paralysis
- ● 6 - Endurance
- ● 7 - Ambulation
- ○ 8 - Speech
- ○ 9 - Legally blind
- ○ A - Dyspnea with minimal exertion

Dyspnea with moderate exertion

Emergency Preparedness (POC Element):

Discussed the HHA's plans for supporting their patients during a natural or man-made disaster
Notified local utility companies and local emergency offices notified of life supporting equipment being used
Discussed the development of the patient's individualized emergency preparedness plan of care, including self-care readiness and the procedure to follow up with the HHA in the event services are interrupted
Discussed patient specific emergency planning options
Educational materials provided to suggest/assist with emergency management/decision making priorities
List of local and state approved evacuation routes and community shelters relevant to the patient's specific geographic location
Obtained the patient's emergency contact numbers for the medical record
Written materials to restate/reinforce the emergency preparedness procedures given to the Patient, and Representative.
List of recommended items to have prepared/ready and available in the event of an emergency

Acuity Level:

Acuity Level: **Medium Risk or Medium Priority**

Emergency Triage:

Emergency Triage: **Category 3 - Visit could be postponed 24-48 hours without adverse effects**

Additional Emergency Preparedness Information:

- [x] Needs assistance during an emergency
- [x] Contact made with local/state emergency preparedness officials regarding patient in need of help during an evacuation
- [x] Medical Needs/Equipment (i.e. bedbound, oxygen, vent, IV cardiac meds other DME)

Comments:

Advance Directives (POC Element (§484.60 (2.xv))):

Patient has Advance Directives? ○ Yes ○ No

If you have checked "yes", fill the following fields:

- [ ] Healthcare Representative
- [ ] Do Not Resuscitate
- [ ] Power of Attorney
- [ ] Living Will
- [ ] The patient has not executed any advance directives to health care.
- [ ] Conservator Assigned
- [ ] Organ Donor
- [ ] Funeral Arrangements Made
- [x] Full Code — Copies to Follow
- [x] The patient was provided with written and verbal information on advance directives.

Comments:

Has surrogate? ○ Yes ○ No

Surrogate Name:

Surrogate Phone:

Health Screening:

Last Cholesterol Level: mm/dd/yyyy

Last Mammogram: mm/dd/yyyy

Does patient perform monthly self breast exam? ○ Yes ○ No

Last Pap Smear: mm/dd/yyyy

Last PSA: mm/dd/yyyy

Last Prostate Exam: mm/dd/yyyy

Last Colonoscopy: mm/dd/yyyy

ACTIVE DIAGNOSES

(M1021/1023) Diagnoses and Symptom Control (POC Element (§484.60 (2.i))):

List each diagnosis for which the patient is receiving home care in Column 1, and enter its ICD-10-CM code at the level of highest specificity in Column 2 (diagnosis codes only – no surgical or procedure codes allowed). Diagnoses are listed in the order that best reflects the seriousness of each condition and supports the disciplines and services provided. Rate the degree of symptom control for each condition in Column 2. ICD-10-CM sequencing requirements must be followed if multiple coding is indicated for any diagnoses.

Code each row according to the following directions for each column:

- **Column** 1: Enter the description of the diagnosis. Sequencing of diagnoses should reflect the seriousness of each condition and support the disciplines and services provided. Column 2: Enter the ICD-10-CM code for the condition described in Column 1 – no surgical or procedure codes allowed. Codes must be entered at the level of highest specificity and ICD-10-CM coding rules and sequencing requirements must be followed. Note that external cause codes (ICD-10-CM codes beginning with V, W, X, or Y) may not be reported in M1021 (Primary Diagnosis) but may be reported in M1023 (Secondary Diagnoses). Also note that when a Z-code is reported in

- **Column 2 :** the code for the underlying condition can often be entered in Column 2, as long as it is an active on-going condition impacting home health care. Rate the degree of symptom control for the condition listed in Column 1. Do not assign a symptom control rating if the diagnosis code is a V, W, X, Y or Z-code. Choose one value that represents the degree of symptom control appropriate for each diagnosis using the following scale: 0 - Asymptomatic, no treatment needed at this time 1 - Symptoms well controlled with current therapy 2 - Symptoms controlled with difficulty, affecting daily functioning; patient needs ongoing monitoring 3 - Symptoms poorly controlled; patient needs frequent adjustment in treatment and dose monitoring 4 - Symptoms poorly controlled; history of re-hospitalizations Note that the rating for symptom control in Column 2 should not be used to determine the sequencing of the diagnoses listed in Column 1. These are separate items and sequencing may not coincide.

	Onset Date	Severity	
PRIMARY DIAGNOSIS I69.351 - Hemiplga following cerebral infrc aff right dominant side	04/16/2026	0 ○ 1 ○ 2 ● 3 ○ 4 ○ ^ ○ Onset ○ Exacerbate ●	
OTHER DIAGNOSIS - 1 I10. - Essential (primary) hypertension	04/16/2026	0 ○ 1 ○ 2 ● 3 ○ 4 ○ ^ ○ Onset ● Exacerbate ○	
OTHER DIAGNOSIS - 2 G62.9 - Polyneuropathy, unspecified	04/16/2026	0 ○ 1 ○ 2 ● 3 ○ 4 ○ ^ ○ Onset ● Exacerbate ○	
OTHER DIAGNOSIS - 3 N50.89 - Other specified disorders of the male genital organs	04/16/2026	0 ○ 1 ○ 2 ● 3 ○ 4 ○ ^ ○ Onset ● Exacerbate ○	
OTHER DIAGNOSIS - 4 K59.00 - Constipation, unspecified	04/16/2026	0 ○ 1 ○ 2 ● 3 ○ 4 ○ ^ ○ Onset ● Exacerbate ○	
OTHER DIAGNOSIS - 5 H53.8 - Other visual disturbances	04/16/2026	0 ○ 1 ○ 2 ● 3 ○ 4 ○ ^ ○ Onset ● Exacerbate ○	

(M1028) Active Diagnoses:

Comorbidities and Co-existing Conditions *– Check all that apply See OASIS Guidance Manual for a complete list of relevant ICD-10 codes.*

- [ ] **1. Peripheral Vascular Disease (PVD) or Peripheral Arterial Disease (PAD)**
- [ ] **2. Diabetes Mellitus (DM)**
- [x] **3. None of the above**

VITAL SIGNS & PAIN ASSESSMENT

(M1060) Height and Weight - While measuring, if the number is X.1-X.4 round down; X.5 or greater round up:

A. Height (in inches). Record most recent height measure since the most recent SOC/ROC:
74

B. Weight (in pounds). Base weight on most recent measure in last 30 days; measure weight consistently, according to standard agency practice (for example, in a.m. after voiding, before meal, with shoes off, etc.:
174

BMI:
22.34

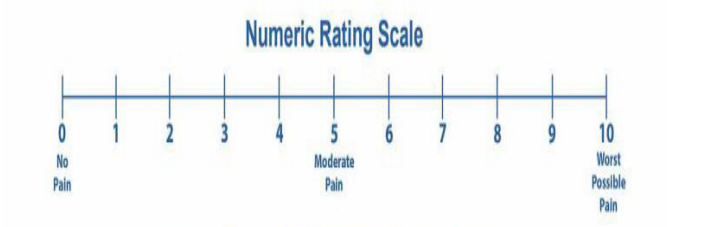
Comments:

Vital Signs

Temperature:
Respiratory (per minute): 18
Blood Pressure mm/Hg (R): /
Blood Sugar (mg/dl): Non-Fasting
O2 Sat: 97
Comments:

Pulse (per minute): Apical = 74 Radial =
Weight: 174 reported
Blood Pressure mm/Hg (L): 110 / 60 sitting

O2 Sat in: Room Air



4

Collected Using: ☑ Face Scale ☑ Numeric Rating Scale

Pain Location and Description:

1 **Location**
Right shoulder and right foot

Start Date
04/16/2026

Activity
With Activity×

What makes pain worse?
Movement×

Intensity
4×

Duration
Days

What makes pain better?
Med/Rest×

Does pain affect quality of life?
Yes

Unable to Assess ☐

Unable to Assess ☐

Description
Dull Ache×

Frequency
Occasional×

Type of Intervention
Non Opiod Analgesic×

How well it is working
Fairly Well×

Status
Treat×

Comments:

(J0510) Pain Effect on Sleep:

Ask patient: *"Over the past 5 days, how much of the time has pain made it hard for you to sleep at night"*

- ○ **0. Does not apply – I have not had any pain or hurting in the past 5 days**
- ○ **1. Rarely or not at all**
- ● **2. Occasionally**
- ○ **3. Frequently**
- ○ **4. Almost constantly**
- ○ **8. Unable to answer**

(J0520) Pain Interference with Therapy Activities:

Ask patient: *"Over the past 5 days, how often have you limited your participation in rehabilitation therapy sessions due to pain?"*

- ● **0. Does not apply – I have not received rehabilitation therapy in the past 5 days**
- ○ **1. Rarely or not at all**
- ○ **2. Occasionally**
- ○ **3. Frequently**
- ○ **4. Almost constantly**
- ○ **8. Unable to answer**

(J0530) Pain Interference with Day-to-Day Activities:

Ask patient: *"Over the past 5 days, how often you have limited your day to day activities (excluding rehabilitation therapy session) because of pain?"*

○ **1. Rarely or not at all** ● **2. Occasionally** ○ **3. Frequently** ○ **4. Almost Constantly** ○ **8. Unable to answer**

MEDICATION & ALLERGIES (INJECTABLE MEDICATIONS)

Medication (POC Element (§484.60 (2.x))):

☐	Start Date	Medication	Strength / Dosage / Frequency	Route	Classification/Indication	Status

Medication Remarks
Medication remarks...

☐ Discontinued Medications

Start Date	Medication	Strength / Dose / Frequency / Route	Classification/Indication	Laboratory	Status	Disc Date	Remarks

Allergies (POC Element):

☐ Add Allergies

Name	Reaction	Start Date	
Amlodipine		mm/dd/yyyy	☐
Morphine		mm/dd/yyyy	☐
Penicillin		mm/dd/yyyy	☐

(M2001) Drug Regimen Review:

Did a complete drug regimen review identify potential clinically significant medication issues?

- [x] 0. No – No issues found during review [Go to M2010, Patient/Caregiver High-Risk Drug Education]
- [ ] 1. Yes – Issues found during review
- [ ] Not assessed/no information
- [ ] 9. NA – Patient is not taking any medications [Go to O0110, Special Treatments, Procedures, and Programs]

(M2003) Medication Follow-up:

Did the agency contact a physician (or physician designee) by midnight of the next calendar day and complete prescribed/recommended actions in response to the identified potential clinically significant medication issues?

- [ ] 0. No
- [ ] 1. Yes
- [ ] Not assessed/no information

(N0415) High-Risk Drug Classes: Use and Indication:

	1. Is Taking	2. Indication Noted
	1. Is taking Check if the patient is taking any medications by pharmacological classification, not how it is used, in the following classes	2. Indication noted If Column 1 is checked, check if there is an indication noted for all medications in the drug class
A. Antipsychotic	[ ]	[ ]
E. Anticoagulant	[ ]	[ ]
F. Antibiotic	[ ]	[ ]
H. Opioid	[ ]	[ ]
I. Antiplatelet	[ ]	[ ]
J. Hypoglycemic (including insulin)	[ ]	[ ]
Z. None of the Above	[x]	

(M2010) Patient/Caregiver High-Risk Drug Education:

Has the patient/caregiver received instruction on special precautions for all high risk medications (such as hypoglycemics, anticoagulants, etc.) and how and when to report problems that may occur?

- [ ] 0. No
- [x] 1. Yes
- [ ] NA - Patient not taking any high-risk drugs OR patient/caregiver fully knowledgeable about special precautions associated with all high-risk medications

(M2020) Management of Oral Medications:

Patient's current ability to prepare and take all oral medications reliably and safely, including administration of the correct dosage at the appropriate times/intervals. Excludes injectable and IV medications. ***(NOTE: This refers to ability, not compliance or willingness.)***

- [ ] 0. Able to independently take the correct oral medication(s) and proper dosage(s) at the correct times.
- [ ] 1. Able to take medication(s) at the correct times if: (a) individual dosages are prepared in advance by another person; OR (b) another person develops a drug diary or chart.
- [x] 2. Able to take medication(s) at the correct times if given reminders by another person at the appropriate times.
- [ ] 3. Unable to take medication unless administered by another person
- [ ] NA - No oral medications prescribed

(M2030) Management of Injectable Medications:

Patient's current ability to prepare and take all prescribed injectable medications reliably and safely, including administration of correct dosage at the appropriate times/intervals. Excludes IV medications.

- [ ] 0. Able to independently take the correct medication(s) and proper dosage(s) at the correct times
- [ ] 1. Able to take injectable medication(s) at the correct times if: a. individual syringes are prepared in advance by another person; OR b. another person develops a drug diary or chart.
- [ ] 2. Able to take medication(s) at the correct times if given reminders by another person based on the frequency of the injection
- [ ] 3. Unable to take injectable medication unless administered by another person
- [x] NA - No injectable medications prescribed.

(O0110) Special Treatments, Procedures, and Programs:

Check all of the following treatments, procedures, and programs that apply at discharge.

	On Admission (Check All That Apply)
Cancer Treatment	
A1. Chemotherapy	[ ]
A2. IV	[ ]
A3. Oral	[ ]
A10. Other	[ ]
B1. Radiation	[ ]
Respiratory Therapies	
C1. Oxygen Therapy	[ ]
C2. Continuous	[ ]
C3. Intermittent	[ ]
C4. High-Concentration	[ ]
D1. Suctioning	[ ]
D2. Scheduled	[ ]
D3. As needed	[ ]
E1. Tracheostomy Care	[ ]
F1. Invasive Mechanical Ventilator	[ ]
G1. Non-invasive Mech. Vent.	[ ]
G2. BiPAP	[ ]
G3. CPAP	[ ]
Others	
H1. IV Medications	[ ]
H2. Vasoactive medications	[ ]
H3. Antibiotics	[ ]

H4. Anticoagulation	☐
H10. Other	☐
I1. Transfusions	☐
J1. Dialysis	☐
J2. Hemodialysis	☐
J3. Peritoneal dialysis	☐
O1. IV Access	☐
O2. Peripheral	☐
O3. Mid-line	☐
O4. Central (e.g., PICC, tunneled, port)	☐
Z1. None of the Above	☑

Most Recent Immunizations:

Pneumonia:	**Flu:**	**Tetanus:**	**TB:**
☐ Yes ☐ No ☑ Unknown	☐ Yes ☐ No ☑ Unknown	☐ Yes ☐ No ☑ Unknown	☐ Yes ☐ No ☑ Unknown
mm/dd/yyyy	mm/dd/yyyy	mm/dd/yyyy	mm/dd/yyyy
TB Exposure:	**Hepatitis B:**	**COVID-19:**	**Additional Immunization:**
☐ Yes ☐ No ☑ Unknown	☐ Yes ☐ No ☑ Unknown	☐ Yes ☐ No ☑ Unknown	☐ Yes ☐ No ☐ Unknown
mm/dd/yyyy	mm/dd/yyyy	mm/dd/yyyy	mm/dd/yyyy

Comments:
Patient and caregiver unable to find the paper for immunization. Taught patient/caregiver the importance of up-to-date immunizations.

NEUROLOGICAL (HEAD, MOOD, EYES, EARS)

Neuro/Emotional/Behavioral Assessment

Neuromuscular:
☐ **No Problems Identified**

Level of Consciousness:
☑ **Awake** ☑ **Alert** ☐ **Lethargic** ☐ **Minimally Responsive** ☐ **Unresponsive** ☐ **Coma**

Impairment:
☐ **Speech** ☐ **Hearing** ☑ **Visual** ☐ **Tremors** ☑ **Numbness** ☑ **Tingling** ☐ **Vertigo** ☐ **Ataxia** ☐ **Decreased Sensitivity**

Speech Impairment (Specify):

Psychosocial (POC Element (§484.60 (2.ii))):
☐ **Document as Clinical Narrative**

Educational level

Primary Language
English

Unable to: ☐ *Read* ☐ *Write*

Barriers:

Language
○ **Needs interpreter**

Learning
○ **Mental**
○ **Physical**
○ **Psychosocial**
○ **Functional**

Emotions Reported:

○ **Angry**
○ **Discouraged**
○ **Withdrawn**

○ **Flat affect**
○ **Difficulty coping**
○ **Disorganized**

○ **Depressed** ○ **Recent** ○ **Long Term**

Treatment:

Evidence of abuse/neglect/exploitation:

○ **Potential**
○ **Verbal/Emotional**
○ **Physical**

○ **Actual**
○ **Financial**
● **No Evidence of Abuse/Neglect/Exploitation**

Unable to cope with altered health status, patient:

○ **Lacks motivation** ○ **Has unrealistic expectations** ● **Denies problems** ○ **Unable to recognize problems**

Sleep/Rest

● **Adequate** ○ **Inadequate**

No. of hours slept per night:
7

Comments

Spiritual / Cultural implications that impact care

Spiritual resource

☐ Inappropriate responses to caregivers/clinician

☐ Inappropriate follow-through in past

Did patient drive a vehicle before admission? ☐Yes ☐No ☑NA

Did the patient have a job before admission? ○Yes ◉No

If yes, do they want to drive post-discharge? ☐Yes ☐No

If yes, do they want to return to work post-discharge? ○Yes ○No

(C0100) Should Brief Interview for Mental Status (C0200-C0500) be Conducted?

Attempt to conduct interview with all patients. ○ **No (patient is rarely/never understood)** ◉ **Yes** ○ **Not assessed/no information**

Brief Interview for Mental Status (BIMS)

(C0200) Repetition of Three Words

Ask patient: **"I am going to say three words for you to remember. Please repeat the words after I have said all three. The words are: SOCK , BLUE , and BED . Now tell me the three words."**

Number of words repeated after first attempt ○ **None** ○ **One** ◉ **Two** ○ **Three** ○ **Not Assessed**

After the patient's first attempt, repeat the words using cues (**"sock, something to wear; blue, a color; bed, a piece of furniture"**). You may repeat the words up to two more times.

(C0300) Temporal Orientation (Orientation to year, month, and day)

Ask patient: **"Please tell me what year it is right now?"**

A) Able to report correct year
- ○ **0. Missed by > 5 years or no answer**
- ○ **1. Missed by 2-5 years**
- ○ **2. Missed by 1 year**
- ◉ **3. Correct**
- ○ **Not assessed/no information**

Ask patient: **"What month are we in right now?"**

B) Able to report correct month
- ○ **0. Missed by > 1 month or no answer**
- ○ **1. Missed by 6 days to 1 month**
- ◉ **2. Accurate within 5 days**
- ○ **Not assessed/no information**

Ask patient: **"What day of the week is today?"**

C) Able to report correct day of the week
- ○ **0. Incorrect or no answer**
- ◉ **1. Correct**
- ○ **Not assessed/no information**

(C0400). Recall

Ask patient: **"Let's go back to an earlier question. What were those three words that I asked you to repeat?" If unable to remember a word, give cue (something to wear; a color; a piece of furniture) for that word."**

A) Able to recall "sock"
- ○ **0. No – could not recall**
- ○ **1. Yes, after cueing ("something to wear")**
- ◉ **2. Yes, no cue required**
- ○ **Not assessed/no information**

B) Able to recall "blue"
- ○ **0. No – could not recall**
- ○ **1. Yes, after cueing ("a color")**
- ◉ **2. Yes, no cue required**
- ○ **Not assessed/no information**

C) Able to recall "bed"
- ○ **0. No – could not recall**
- ○ **1. Yes, after cueing ("a piece of furniture")**
- ◉ **2. Yes, no cue required**
- ○ **Not assessed/no information**

C0500. BIMS Summary Score

Add scores for questions C0200-C0400 and fill in total score (00-15) **Enter 99 if the patient was unable to complete the interview** **14**

(C1310) Signs and Symptoms of Delirium (from CAM©):

A. Acute Onset of Mental Status Change:

Is there evidence of an acute change in mental status from the patient's baseline?

◉ **0. No** ○ **1. Yes**

B. Inattention:

Did the patient have difficulty focusing attention, for example, being easily distractible or having difficulty keeping track of what was being said?

- ○ **0. Behavior not present**
- ◉ **1. Behavior continuously present, does not fluctuate**
- ○ **2. Behavior present, fluctuates (comes and goes, changes in severity)**
- ○ **Not assessed/no information**

C. Disorganized thinking:

Was the patient's thinking disorganized or incoherent (rambling or irrelevant conversation, unclear or illogical flow of ideas, or unpredictable switching from subject to subject)?

- ○ **0. Behavior not present**
- ◉ **1. Behavior continuously present, does not fluctuate**
- ○ **2. Behavior present, fluctuates (comes and goes, changes in severity)**
- ○ **Not assessed/no information**

D. Altered level of consciousness:

Did the patient have altered level of consciousness, as indicated by any of the following criteria?

- ***vigilant*** *– startled easily to any sound or touch* ☐ *lethargic – repeatedly dozed off when being asked questions, but responded to voice or touch*
- ***stuporous*** *– very difficult to arouse and keep aroused for the interview*
- ***comatose*** *– could not be arouse*

- ◉ **0. Behavior not present**
- ○ **1. Behavior continuously present, does not fluctuate**
- ○ **2. Behavior present, fluctuates (comes and goes, changes in severity)**
- ○ **Not assessed/no information**

(D0150) Patient Mood Interview (PHQ-2 to 9):

Say to patient: **"Over the last 2 weeks, have you been bothered by any of the following problems?"**
If symptom is present, **enter 1 (yes) in column 1 , Symptom Presence** . If yes in column 1, then ask the patient: **"About how often have you been bothered by this?"**
Read and show the patient a card with the symptom frequency choices. Indicate response in column 2, Symptom Frequency

1) Symptom Presence
- **0. No (enter 0 in column 2)**
- **1. Yes (enter 0-3 in column 2)**
- **9. No response (leave column 2 blank)**

2) Symptom Frequency
- 0 - **Never or 1 day**
- 1 - **2-6 days (several days)**
- 2 - **7-11 days (half or more of the days)**
- 3 - **12-14 days (nearly every day)**

	Presence	Frequency
A. Little interest or pleasure in doing things	**No** ×	**Never or 1 day**
B. Feeling down, depressed, or hopeless	**No** ×	**Never or 1 day**
If either **D0150A2 or D0150B2** is coded **2 or 3** , **CONTINUE** asking the questions below. If not, **END the PHQ interview**		
C. Trouble falling or staying asleep, or sleeping too much	^	^
D. Feeling tired or having little energy	^	^
E. Poor appetite or overeating	^	^
F. Feeling bad about yourself – or that you are a failure or have let yourself or your family down	^	^
G. Trouble concentrating on things, such as reading the newspaper or watching television	^	^

	Presence	Frequency
H. Moving or speaking so slowly that other people could have noticed. Or the opposite – being so fidgety or restless that you have been moving around a lot more than usual	^	^
I. Thoughts that you would be better off dead, or of hurting yourself in some way	^	^
D0160. Total Severity Score Add scores for all frequency responses in Column 2, Symptom Frequency. Total score must be between 02 and 27. Enter 99 if unable to complete interview (i.e., Symptom Frequency is blank for 3 or more required items)		0

(D0700) Social Isolation:

How often do you feel lonely or isolated from those around you?

- [x] 0. Never
- [ ] 1. Rarely
- [ ] 2. Sometimes
- [ ] 3. Often
- [ ] 4. Always
- [ ] 7. Patient declines to respond
- [ ] 8. Patient unable to respond

(M1740) Cognitive, behavioral, and psychiatric symptoms that are demonstrated at least once a week (Reported or Observed):

(Mark all that apply)

- [ ] 1. Memory deficit: failure to recognize familiar persons/places, inability to recall events of past 24 hours, significant memory loss so that supervision is required
- [x] 2. Impaired decision-making: failure to perform usual ADLs or IADLs, inability to appropriately stop activities, jeopardizes safety through actions
- [ ] 3. Verbal disruption: yelling, threatening, excessive profanity, sexual references, etc.
- [ ] 4. Physical aggression: aggressive or combative to self and others (for example, hits self, throws objects, punches, dangerous maneuvers with wheelchair or other objects)
- [ ] 5. Disruptive, infantile, or socially inappropriate behavior (excludes verbal actions)
- [ ] 6. Delusional, hallucinatory, or paranoid behavior
- [ ] 7. None of the above behaviors demonstrated

(M1745) Frequency of Disruptive Behavior Symptoms (Reported or Observed):

- [ ] 0. Never
- [x] 1. Less than once a month
- [ ] 2. Once a month
- [ ] 3. Several times each month
- [ ] 4. Several times a week
- [ ] 5. At least daily

(M1700) Cognitive Functioning:

Patient's current (day of assessment) level of alertness, orientation, comprehension, concentration, and immediate memory for simple commands

- [ ] 0. Alert/oriented, able to focus and shift attention, comprehends and recalls task directions independently.
- [x] 1. Requires prompting (cuing, repetition, reminders) only under stressful or unfamiliar conditions.
- [ ] 2. Requires assistance and some direction in specific situations (for example, on all tasks involving shifting of attention) or consistently requires low stimulus environment due to distractibility
- [ ] 3. Requires considerable assistance in routine situations. Is not alert and oriented or is unable to shift attention and recall directions more than half the time.
- [ ] 4. Totally dependent due to disturbances such as constant disorientation, coma, persistent vegetative state, or delirium.

(M1710) When Confused (Reported or Observed Within the Last 14 Days):

- [ ] 0. Never
- [x] 1. In new or complex situations only
- [ ] 2. On awakening or at night only
- [ ] 3. During the day and evening, but not constantly
- [ ] 4. Constantly
- [ ] 5. Patient nonresponsive

(M1720) When Anxious (Reported or Observed Within the Last 14 Days):

- [x] 0. None of the time
- [ ] 1. Less often than daily
- [ ] 2. Daily, but not constantly
- [ ] 3. All of the time
- [ ] NA - Patient nonresponsive

Mental Status (POC Element (§484.60 (2.ii)):

- [x] 1 - Oriented
- [ ] 2 - Comatose
- [x] 3 - Forgetful
- [ ] 4 - Depressed
- [ ] 5 - Disoriented
- [ ] 6 - Lethargic
- [ ] 7 - Agitated
- [x] 8 - Alert
- [ ] 9 - Confused
- [ ] 10 - Dementia
- [ ] 11 - Assaultive
- [ ] 12 - Delirious
- [ ] 13 - Wanders
- 14 - Other

Describe the patient's mental status. Include general appearance, behaviors, emotional responses, mental functioning, overall social interaction as well as both clinical objective observations and subjective descriptions reported during this visit.

Patient is alert and oriented with forgetfulness, well-groomed, responds appropriately to questions, follows simple and complex instructions. Patient has normal mood/affect, denies anxiety and depression, at the time of visit. Patient did not express suicidal/homicidal ideation. Patient demonstrated understanding of teachings with teach-back, overall social interaction was within normal limits.

Has there been a sudden/acute change in their mental status? ☐ Yes ☑ No Reported by: ☐ If yes, did the change coincide with something else like medication change, fall, or loss of a loved one? ☐ No ☐ Yes

Comments:

Cultural:

Is religion important to the patient? ◯ Yes ◯ No

Patient's primary source of emotional support:

Eyes and Ears

(B1000) Vision:

Ability to see in adequate light *(with glasses or other visual appliances)*

- [ ] 0. Adequate – sees fine detail, such as regular print in newspapers/books
- [x] 1. Impaired – sees large print, but not regular print in newspapers/books
- [ ] 2. Moderate impaired – limited vision; not able to see newspaper headlines but can identify objects
- [ ] 3. Highly impaired – object identification in question, but eyes appear to follow objects
- [ ] 4. Severely impaired – no vision or sees only light, colors or shapes; eyes do not appear to follow objects

Vision Assessment:

- [ ] No Problems Identified
- [ ] Glasses
- [ ] Contact Lenses: [ ] Right [ ] Left
- [ ] Glaucoma: [ ] Right [ ] Left
- [x] Blurred Vision: [x] Right [x] Left
- [ ] Macular Degeneration: [ ] Right [ ] Left
- [ ] Blindness: [ ] Right [ ] Left
- [ ] Cataracts: [ ] Right [ ] Left
- [ ] Other (Specify):

Pupils:

- [x] PERRLA
- [ ] Anisocoria: [ ] Right [ ] Left
- [ ] Other (Specify):

Comments:

(B0200) Hearing:

Ability to hear *(with hearing aid or hearing appliances if normally used)*

- [x] 0. Adequate – no difficulty in normal conversation, social interaction, listening to TV
- [ ] 1. Minimal difficulty – difficulty in some environments (e.g., when person speaks softly, or setting is noisy)
- [ ] 2. Moderate difficulty – speaker has to increase volume and speak distinctly
- [ ] 3. Highly impaired – absence of useful hearing

Hearing Assessment:

- [x] No Problems Identified
- [ ] Right Hearing Impairment
- [ ] Left Hearing Impairment
- [ ] Deaf
- [ ] Hearing Aids:
- [ ] Right
- [ ] Left

Comments:

CARDIOPULMONARY (CHEST & THORAX)

Heart Sounds:

- [x] No Problems Identified
- [ ] Irregular: [ ] Known Hx [ ] New Finding
- [ ] Gallops: [ ] Known Hx [ ] New Finding
- [ ] Clicks: [ ] Known Hx [ ] New Finding
- [ ] Murmur: [ ] Known Hx [ ] New Finding
- [ ] FrictionRub: [ ] Known Hx [ ] New Finding

Comments:

Edema:

- [ ] Rigth Upper Extremities:
- [ ] Left Upper Extremities:
- [ ] Rigth Lower Extremities:
- [ ] Left Lower Extremities:

Others:

Others:

Comments:

Peripheral Pulses:

Radial: [x] Strong [ ] Bounding [ ] Weak [ ] Thready [ ] Absent

Pedal: [x] Strong [ ] Bounding [ ] Weak [ ] Thready [ ] Absent

Fluid Imbalance and Systemic Distress:

- [ ] Cramps
- [ ] Claudication
- [ ] Fatigue
- [ ] Face
- [ ] Neck
- [ ] Abdomen

Capillary Refill:

- [x] < 3 secs
- [ ] > 3 secs

Chest/Thorax

Chest Pain:

- [x] Denies
- [ ] Anginal
- [ ] Postural
- [ ] Localized
- [ ] Substernal
- [ ] Sharp
- [ ] Dull
- [ ] Aching
- [ ] Radiating
- [ ] Viselike
- [ ] Stabbing
- [ ] Nausea
- [ ] Other (Specify):

Associated with:

- [ ] Shortness of Breath
- [ ] Activity
- [ ] Rest
- [ ] Diaphoresis
- [ ] Frequency/Duration:

(M1400) When is the patient dyspneic or noticeably Short of Breath?

- [ ] 0. Patient is not short of breath
- [ ] 1. When walking more than 20 feet, climbing stairs
- [x] 2. With moderate exertion (for example, while dressing, using commode or bedpan, walking distances less than 20 feet)
- [ ] 3. With minimal exertion (for example, while eating, talking, or performing other ADLs) or with agitation
- [ ] 4. At rest (during day or night)

Respiratory Status:

Respiratory Distress Indicators:

- [x] Dyspnea:
- [ ] At Rest
- [x] With Exertion/Activity
- [ ] Accessory Muscle Used
- [ ] Orthopnea
- [ ] Retraction
- [ ] Stridor
- [ ] O2 Liters per minute:
- [ ] O2 Saturation (%):

Frequency/Timing of Oxygen Use:

- [ ] PRN
- [ ] Continuous
- [ ] Only at bedtime
- [ ] Other (Specify):

Route/Delivery:

- [ ] Nasal Cannula
- [ ] Trach Cuff
- [ ] Mask (Specify):
- [ ] Other (Specify):

Lung Sounds:

Anterior Right:
Anterior Right: **Clear**

Anterior Left:
Anterior Left: **Clear**

Posterior Upper Right:
Posterior Upper Right: **Clear**

Posterior Upper Left:
Posterior Upper Left: **Clear**

Posterior Lower Right:
Posterior Lower Right: **Clear**

Posterior Lower Left:
Posterior Lower Left: **Clear**

Comments:

Cough:

Cough Characteristics:

- [x] None
- [ ] Dry
- [ ] Acute
- [ ] Chronic

Productivity of Cough:

- [ ] Non-Productive
- [ ] Productive:

Sputum Characteristics (if Productive):

- [ ] Brown
- [ ] Clear
- [ ] Cloudy
- [ ] Bloody
- [ ] Yellow
- [ ] White
- [ ] Hemoptysis:
- [ ] Amount (Specify):
- [ ] Frequency (Specify):

Gastrointestinal (Mouth - Anus)

Mouth and Throat (Assess oral health and swallowing ability):

- [ ] Dentures: [ ] Upper [ ] Lower [ ] Partial
- [ ] Bleeding Gums/Gingivitis [ ] Toothache [ ] Ulcerations [ ] Lesion
- [ ] Dysphagia [ ] Hoarseness [ ] Sore Throat [ ] Masses/Tumors

Comments:

(M1870) Feeding or Eating:

*Current ability to feed self meals and snacks safely. Note: This refers only to the process of **eating**, **chewing**, and **swallowing**, not preparing the food to be eaten.*

- ( ) 0. Able to independently feed self.
- (✓) 1. Able to feed self independently but requires: (a) meal set-up; OR (b) intermittent assistance or supervision from another person; OR (c) a liquid, pureed or ground meat diet.
- ( ) 2. Unable to feed self and must be assisted or supervised throughout the meal/snack.
- ( ) 3. Able to take in nutrients orally and receives supplemental nutrients though a nasogastric tube or gastrostomy.
- ( ) 4. Unable to take in nutrients orally and is fed nutrients through a nasogastric tube or gastrostomy.
- ( ) 5. Unable to take in nutrients orally or by tube feeding.

Appetite (%): **Appetite Percentage of (Meals per day):** **Snacks (Per day):**

Appetite: [ ] Increase [ ] Decreased [✓] Unchanged

Intake: [✓] Adequate [ ] Inadequate

Nutritional Abnormalities:

[ ] Anoxeria [ ] Cachexia [ ] Decreased Appetite [✓] Constipation

(K0520) Nutritional Approaches:

	On Admission (Check All That Apply)
A. Parenteral/IV feeding	[ ]
B. Feeding tube (e.g., nasogastric or abdominal (PEG))	[ ]
C. Mechanically altered diet – require change in texture of food or liquids (e.g., pureed food, thickened liquids)	[ ]
D. Therapeutic diet (e.g., low salt, diabetic, low cholesterol)	[✓]
Z. None of the above	[ ]

#16 Nutritional Requirements (POC Element (§484.60 (2.ix))):

[ ] NPO [ ] No Added Salt [✓] Low Salt [✓] Low Fat [ ] Low Cholesterol [ ] No Concentrated Sweets
[ ] Controlled Carbohydrates [ ] Regular [ ] Fluid Restriction:
Other:

Nutritional Health Screen	Score	Nutritional Health Screen	Score
( ) Without reason, has lost more than 10 lbs in the last 3 months	15	( ) Does not always have enough money to buy foods need	10
(●) Has an illness or condition that made pt change the type and/or amount of food eaten	10	( ) Eats alone most of the time	5
( ) Has open decubitus, ulcer, burn or wound	10		
( ) Eats fewer than 2 meals a day	10	(●) Takes 3 or more prescribed or OTC medications a day	5
( ) Has a tooth/mouth problem that makes it hard to eat	10	( ) Is not always physically able to cook and/or feed self and has no caregiver to assist	5
( ) Has 3 or more drinks of beer, liquor or wine almost every day	10	(●) Frequently has a diarrhea or constipation	5
		Total	20

(●) Good Nutritional Status (Score 0 - 25) ( ) Moderate Nutritional Status (Score 25 - 54) ( ) High Nutritional Status (Score 55 - 100)

Nutritional Status Commments:

Weight:

[✓] No Change [ ] Weight Gain [ ] Weight Loss

Comments:

(M1620) Bowel Incontinence Frequency:

- (●) 0. Very rarely or never has bowel incontinence
- ( ) 1. Less than once weekly
- ( ) 2. One to three times weekly
- ( ) 3. Four to six times weekly
- ( ) 4. On a daily basis
- ( ) 5. More often than once daily
- ( ) NA - Patient has ostomy for bowel elimination
- ( ) UK - Unknown

(M1630) Ostomy for Bowel Elimination:

Does this patient have an ostomy for bowel elimination that (within the last 14 days): a) was related to an inpatient facility stay; or b) necessitated a change in medical or treatment regimen?

- (●) 0. Patient does not have an ostomy for bowel elimination
- ( ) 1. Patient's ostomy was not related to an inpatient stay and did not necessitate change in medical or treatment regimen.
- ( ) 2. The ostomy was related to an inpatient stay or did necessitate change in medical or treatment regimen.

Bowel Sounds (Abdominal Quadrants):

Right Lower:	Right Upper:	Left Lower:	Left Upper:	Last Bowel Movement:
Right Lower: **Normoactive**	Right Upper: **Normoactive**	Left Lower: **Normoactive**	Left Upper: **Normoactive**	04/15/2026

GI Symptoms:

☐ Nausea ☐ Vomiting ☐ Reflux/Indigestion ☐ Bowel Incontinence ☐ Diarrhea ☑ Constipation

Duration (If Constipation is checked):

☑ Chronic ☐ Acute ☐ Occasional

Abdominal Assessment:

☐ Abdominal Soft/Non-tender ☑ Abdominal Soft/Tender ☐ Abdominal Distention ☐ Girth(cm):

Hemorrhoids:

☐ External Hemorrhoids ☐ Internal Hemorrhoids ☐ Laxative/Enema Use

Stool Appearance:

☐ Gray ☐ Tarry ☐ Brown/Tan ☐ Black ☐ Fresh Blood ☐ Other (Specify):

Ostomy/Stoma (if applicable):

☐ Moist ☐ Dry ☐ Pinkished-Red ☐ Purple ☐ Gray ☐ Black ☐ Odor

Peristomal Skin Characteristics:

☐ Clean ☐ Healthy ☐ Red ☐ Weeping ☐ Peeling ☐ Inflamed ☐ Other (Specify):

Comments:

Genitourinary (Genitals & Urinary)

(M1600) Has this patient been treated for a Urinary Tract Infection in the past 14 days?

● 0. No ○ 1. Yes ○ NA - Patient on prophylactic treatment ○ UK - Unknown

(M1610) Urinary Incontinence or Urinary Catheter Presence:

● Patient is incontinent
- ○ Day time
- ○ Night time
- ○ Day and Night
- ○ Timed-voiding defers incontinence
- ● Occasional stress incontinence

○ No incontinence or catheter (includes anuria or ostomy for urinary drainage)

○ Patient requires a urinary catheter (specifically: external, indwelling, intermittent, or suprapubic)

Genitourinary Assessment:

☐ No Problems Identified

Abnormalities:

☐ Frequency ☐ Dysuria ☐ Urgency ☐ Anuria ☐ Nocturia ☐ Oliguria
☐ Retention ☐ Hematuria ☐ Irrigation ☑ Incontinence ☐ Diapers Used ☐ Penile Discharge
☐ Vaginal Bleeding/Discharge ☐ Other (Specify):

Urine Color:

☑ Transparent ☑ Pale Yellow ☐ Brownish Orange ☐ Pinkish Red
☐ Dark Yellow ☐ Blue or Green ☐ Foamy ☐ Transparent Yellow

Urine Consistency:

☑ Clear ☐ Sediment ☐ Odorous ☐ Cloudy

Urinary Catheter:

☐ Yes ☑ No

☐ External Catheter ☐ Foley ☐ Suprapubic ☐ Nephrostomy

☐ Last Catheter Change: mm/dd/yyyy ☐ Fr: ☐ cc:

Comments:

INTEGUMENTARY (SKIN & WOUND)

(M1306) Does this patient have at least one Unhealed Pressure Ulcer/Injury at Stage 2 or Higher or designated as Unstageable?

(Excludes Stage 1 pressure injuries and all healed pressure ulcers/injuries)

● 0. No **[→ Skip to M1322, Current Number of Stage 1 Pressure Injuries at SOC/ROC; Skip to M1324, Stage of Most Problematic Unhealed Pressure Ulcer/Injury that is Stageable at DC]**

○ 1. Yes

(M1311) Current Number of Unhealed Pressure Ulcers/Injuries at Each Stage:

A1. Stage 2: Partial thickness loss of dermis presenting as a shallow open ulcer with a red or pink wound bed, without slough. May also present as an intact or open/ruptured blister. **Number of Stage 2 pressure ulcers**	^
B1. Stage 3: Full thickness tissue loss. Subcutaneous fat may be visible but bone, tendon, or muscle is not exposed. Slough may be present but does not obscure the depth of tissue loss. May include undermining and tunneling. **Number of Stage 3 pressure ulcers**	^
C1. Stage 4: Full thickness tissue loss with exposed bone, tendon, or muscle. Slough or eschar may be present on some parts of the wound bed. Often includes undermining and tunneling. **Number of Stage 4 pressure ulcers**	^
D1. Unstageable: Non-removable dressing/device: Known but not stageable due to non-removable dressing/device **Number of unstageable pressure ulcers/injuries due to non-removable dressing/device**	^
E1. Unstageable: Slough and/or eschar: Known but not stageable due to coverage of wound bed by slough and/or eschar **Number of unstageable pressure ulcers due to coverage of wound bed by slough and/or eschar**	^
F1. Unstageable: Deep tissue injury Number of unstageable pressure injuries presenting as deep tissue injury	^

(M1322) Current Number of Stage 1 Pressure Injuries:

Intact skin with non blanchable redness of a localized area usually over a bony prominence. Darkly pigmented skin may not have a visible blanching; in dark skin tones only it may appear with persis tent blue or purple hues.

Stage 1 Pressure Injuries: **0**

(M1324) Stage of Most Problematic Unhealed Pressure Ulcer/Injury that is Stageable:

(Excludes pressure ulcer/injury that cannot be staged due to a non removable dressing/device, coverage of wound bed by slough and/or eschar, or deep tissue injury.)

Stage of Most Problematic: **Patient has no pressure ulcers or no stageable pressure ulcers**

(M1330) Does this patient have a Stasis Ulcer?

● 0. No **[GO TO M1340]**

○ 1. Yes, patient has BOTH observable and unobservable stasis ulcers

- ◯ 2. Yes, patient has observable stasis ulcers ONLY
- ◯ 3. Yes, patient has unobservable stasis ulcers ONLY (known but not observable due to non-removable dressing/device) _ **[GO TO M1340]**

(M1332) Current Number of Stasis Ulcer(s) that are Observable:

◯ 1. One ◯ 2. Two ◯ 3. Three ◯ 4. Four or more

(M1334) Status of Most Problematic Stasis Ulcer that is Observable:

◯ 1. Fully granulating ◯ 2. Early/partial granulation ◯ 3. Not healing

(M1340) Does this patient have a Surgical Wound?

- ● 0. No
- ◯ 1. Yes, patient has at least one observable surgical wound
- ◯ 2. Surgical wound known but not observable due to non-removable dressing/device

(M1342) Status of Most Problematic Surgical Wound that is Observable:

◯ 0. Newly epithelialized ◯ 1. Fully granulating ◯ 2. Early/partial granulation ◯ 3. Not healing

Skin Color:

☑ Normal ☐ Pale ☐ Jaundice ☐ Cyanotic

Skin Turgor:

☑ Good ☐ Fair ☐ Poor

Skin Abnormalities:

☑ Dry ☑ Warm ☐ Cool ☐ Rash ☐ Ulcer ☐ Ostomy ☐ Incision ☐ Diaphoretic ☐ Other (Specify):

Comments:

Norton Scale for Pressure Ulcer Risk Assessment:

	Good (4)	Fair (3)	Poor (2)	Very Bad (1)
Physical Condition	◯ Good	● Fair	◯ Poor	◯ Very Bad
Mental Condition	● Alert	◯ Apathetic	◯ Confused	◯ Stuporous
Activity	◯ Ambulant	● Walks With Help	◯ Chairbound	◯ Bedfast
Mobility	◯ Full	● Slightly Impaired	◯ Very Limited	◯ Immobile
Incontinence	◯ None	● Occassional	◯ Usually Urinary	◯ Urinary and Fecal
			Total Score:	16

Wound

Wound Worksheet:

Fetch Wounds From Previous Wound Note

SAFETY & RISK ASSESSMENT

Home Environment/Home Safety Assessment

☑ Architectural Barriers	☐ No fire extinguisher	☐ No emergency exit plan
☐ Medications are improperly stored	☐ Inadequate electricity/cooling/heating	☐ Rodent/insect infestation
☐ No telephone	☐ Inadequate supply of water	☐ Unsafe electrical wiring
☑ Cluttered surroundings	☐ Inadequate sanitation/plumbing	☐ Nonworking stove/refrigerator
☑ Inadequate railing	☐ Inadequate smoke detectors on all levels of home	☐ Exposed flammables
☐ Unsafe oxygen use	☐ Unsafe storage of equipments	☑ Inadequate Lighting

Others (Specify):

Type of Assistance Patient Receives - other than from Home Health Agency Staff:

ADL (Bathing, dressing, toileting, eating/feeding, bowel):

☑ Family/Friends ☐ Provider Services ☐ Paid Caregiver ☐ Volunteer Organizations

IADL (Meds, meals, house-keeping, laundry, telephone, shipping, finances):

☑ Family/Friends ☐ Provider Services ☐ Paid Caregiver ☐ Volunteer Organizations

Psychosocial Support:

☑ Family/Friends ☐ Provider Services ☐ Paid Caregiver ☐ Volunteer Organizations

Assistance with doctors appointment, delivery of medications:

☑ Family/Friends ☐ Provider Services ☐ Paid Caregiver ☐ Volunteer Organizations

Management of Finances:

☑ Family/Friends ☐ Provider Services ☐ Paid Caregiver ☐ Volunteer Organizations

Comments:

(M1100) Patient Living Situation. Which of the following best describes the patient's residential circumstance and availability of assistance?

Living Arrangement	Around the clock	Regular daytime	Regular Night time	Short-term Assistance	No Assistance available
A. Patient lives alone	◯ Patient Lives alone around the clock	◯ Patient Lives alone Regular daytime	◯ Patient Lives alone Regular Night time	◯ Patient Lives alone Short-term Assistance	◯ Patient Lives alone No Assistance available
B. Patient lives with other person(s) in the home	● Patient lives with other person(s) in the home Around the clock	◯ Patient lives with other person(s) in the home Regular daytime	◯ Patient lives with other person(s) in the home Regular Night time	◯ Patient lives with other person(s) in the home Short-term Assistance	◯ Patient lives with other person(s) in the home No Assistance available
C. Patient lives in congregate situation (for example, assisted living, residential care home)	◯ Patient lives in congregate situation (for example, assisted living, residential care home) around the clock	◯ Regular daytime	◯ Patient lives in congregate situation Regular Night time	◯ Patient lives in congregate situation Short-term Assistance	◯ Patient lives in congregate situation No Assistance available

(M2102) Types and Sources of Assistance

Determine the ability and willingness of non agency caregivers (such as family members, friends, or privately paid caregivers) to provide assistance for the following activities, if assistance is needed. Excludes all care by your agency staff

F. Supervision and Safety *(for example, due to cognitive impairment)*

- ◯ 0. No assistance needed – patient is independent or does not have needs in this area

- [x] 1. Non-agency caregiver(s) currently provide assistance
- [ ] 2. Non-agency caregiver(s) need training/supportive services to provide assistance
- [ ] 3. Non-agency caregiver(s) are not likely to provide assistance OR it is unclear if they will provide assistance
- [ ] 4. Assistance needed, but no non-agency caregiver(s) available

Safety Measures (POC Element (§484.60 (2.xi))):

- [ ] 1 - Bleeding precautions
- [ ] 2 - O_2 precautions
- [ ] 3 - Seizure precautions
- [x] 4 - Fall precautions
- [ ] 5 - Aspiration precautions
- [ ] 6 - Siderails up
- [ ] 7 - Elevate head of bed
- [ ] 8 - 24 hr. supervision
- [x] 9- Clear Pathways up
- [x] 10 - Lock w/c with transfers
- [x] 11 - Infection control measures
- [x] 12 - Walker/cane
- [ ] 13 - Sharps Precaution
- [x] 14 - Other:

911/ER protocol, universal precautions

Hospitalization Risk Assessment

(M1033) Risk for Hospitalization:

Which of the following signs or symptoms characterize this patient as at risk for hospitalization? (Mark all that apply.)

- [ ] 1. History of falls (2 or more falls – or any fall with an injury – in the past 12 months)
- [ ] 2. Unintentional weight loss of a total of 10 pounds or more in the past 12 months
- [ ] 3. Multiple hospitalizations (2 or more) in the past 6 months
- [ ] 4. Multiple emergency department visits (2 or more) in the past 6 months
- [ ] 5. Decline in mental, emotional, or behavioral status in the past 3 months
- [ ] 6. Reported or observed history of difficulty complying with any medical instructions (for example, medications, diet, exercise) in the past 3 months
- [x] 7. Currently taking 5 or more medications
- [x] 8. Currently reports exhaustion
- [ ] 9. Other risk(s) not listed in 1 - 8
- [ ] 10. None of the above

Additional Risk Information:

Fall Risk Test

- [ ] N/A
- [x] MAHC 10* - Fall Risk Assessment Tool
- [ ] TUG Assessment Tool

MAHC 10* - FALL RISK ASSESSMENT TOOL

Age 65+: [x] Yes [ ] No

Diagnosis (3 or more co-existing): [x] Yes [ ] No
- *Includes only documented medical diagnosis*

Prior history of falls within 3 months: [ ] Yes [x] No
- *An unintentional change in position resulting in coming to rest on the ground or at a lower level*

Incontinence: [x] Yes [ ] No
- *Inability to make it to the bathroom or commode in timely manner.*
- *Includes frequency, urgency, and/or nocturia.*

Visual impairment: [x] Yes [ ] No
- *Includes but not limited to, macular degeneration, diabetic retinopathies, visual field loss, age related changes, decline in visual acuity, accommodation, glare tol erance, depth perception, and night vision or not wearing prescribed glasses or having the correct prescription.*

Impaired functional mobility: [x] Yes [ ] No
- *May include patients who need help with IADLS or ADLS or have gait or transfer problems, arthritis, pain, fear of falling, foot problems, impaired sensation, imp aired coordination or improper use of assistive devices.*

Environmental hazards: [x] Yes [ ] No
- *May include but not limited to, poor illumination, equipment tubing, inappropriate footwear, pets, hard to reach items, floor surfaces that are uneven or cluttered, or outdoor entry and exits.*

Poly Pharmacy (4 or more prescriptions-any type): [x] Yes [ ] No
- *All PRESCRIPTIONS including prescriptions for OTC meds. Drugs highly associated with fall risk include but not limited to, sedatives, antidepressants, tranquili zers, narcotics, antihypertensives, cardiac meds, corticosteroids, anti-anxiety drugs, anticholinergic drugs, and hypoglycemic drugs.*

Pain affecting level of function: [x] Yes [ ] No
- *Pain often affects an individual's desire or ability to move or pain can be a factor in depression or compliance with safety recommendations.*

Cognitive impairment: [ ] Yes [x] No
- *Could include patients with dementia, Alzheimer's or stroke patients or patients who are confused, use poor judgment, have decreased comprehension, impulsi vity, memory deficits. Consider patients ability to adhere to the plan of care.*

A score of 4 or more is considered at risk for falling: 8.00

Fire Assessment for Patient with Oxygen

- [x] Patient is not using oxygen

FUNCTIONAL ASSESSMENT (SELF CARE)

(GG0100) Prior Functioning: Everyday Activities:

Indicate the patient's usual ability with everyday activities prior to the current illness, exacerbation, or injury.

- ***3. Independent*** *– Patient completed the activities by him/herself, with or without an assistive device, with no assistance from a helper*
- ***2. Needed Some Help*** *– Patient needed partial assistance from another person to complete activities*
- ***1. Dependent*** *– A helper completed the activities for the patient.*
- ***8. Unknown***
- ***9. Not Applicable***

	Independent	Needed Some Help	Dependent	Unknown	Not Applicable
A. Self Care: Code the patient's need for assistance with bathing, dressing, using the toilet, or eating prior to the current illness, exacerbation, or injur	○	✓	○	○	○
B. Indoor Mobility (Ambulation): Code the patient's need for assistance with walking from room to room (with or without a device such as cane, crutch or walker) prior to the current illness, exacerbation, or injury.	○	✓	○	○	○

C. Stairs: Code the patient's need for assistance with internal or external stairs (with or without a device such as cane, crutch, or walker) prior to the current illness, exacerbation or injury.	○	✓	○	○	○
D. Functional Cognition: Code the patient's need for assistance with planning regular tasks, such as shopping or remembering to take medication prior to the current illness, exacerbation, or injury.	✓	○	○	○	○

(GG0110) Prior Device Use:

Indicate devices and aids used by the patient prior to the current illness, exacerbation, or injury ***(Check all that apply)***

- ☐ **A. Manual wheelchair**
- ☑ **B. Motorized wheelchair and/or scooter**
- ☐ **C. Mechanical lift**
- ☐ **D. Walker**
- ☐ **E. Orthotics/Prosthetics**
- ☐ **Z. None of the above**

(GG0130) Self Care

(GG0130) SOC/ROC Performance:

Code the patient's usual performance at SOC/ROC for each activity using the 6-point scale. If activity was not attempted at SOC/ROC, code the reason.

Safety and Quality of Performance – If helper assistance is required because patient's performance is unsafe or of poor quality, score according to amount of assistance provided.

Activities may be completed with or without assistive devices.

06. Independent – Patient completes the activity by him/herself with no assistance from a helper.
05. Setup or clean-up assistance – Helper sets up or cleans up; patient completes activity. Helper assists only prior to or following the activity.
04. Supervision or touching assistance –
Helper provides verbal cues and/or touching/steadying and/or contact guard assistance as patient completes activity. Assistance may be provided throughout the activity or intermittently.
03. Partial/moderate assistance – Helper does LESS THAN HALF the effort. Helper lifts, holds or supports trunk or limbs, but provides less than half the effort.
02. Substantial/maximal assistance – Helper does MORE THAN HALF the effort. Helper lifts or holds trunk or limbs and provides more than half the effort.
01. Dependent – Helper does ALL of the effort. Patient does none of the effort to complete the activity. Or, the assistance of 2 or more helpers is required for the patient to complete the activity.

If activity was not attempted, code reason:

07. Patient refused
09. Not applicable – Not attempted and the patient did not perform this activity prior to the current illness, exacerbation or injury.
10. Not attempted due to environmental limitations (e.g., lack of equipment, weather constraints)
88. Not attempted due to medical conditions or safety concerns

(GG0130A) A. Eating:

- ○ 06. Independent
- ● 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0130B) B. Oral Hygiene:

- ○ 06. Independent
- ● 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0130C) C. Toileting Hygiene:

- ○ 06. Independent
- ● 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0130E) E. Shower/Bathe Self:

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ● 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0130F) F. Upper Body Dressing:

- ○ 06. Independent
- ● 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0130G) G. Lower Body Dressing:

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ● 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0130H) H. Putting on/Taking Off Footwear:

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ● 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(M1800) Grooming:

Current ability to tend safely to personal hygiene needs (specifically: washing face and hands, hair care, shaving or make up, teeth or denture care, or fingernail care)

- ○ **0. Able to groom self unaided, with or without the use of assistive devices or adapted methods.**
- ● **1. Grooming utensils must be placed within reach before able to complete grooming activities.**
- ○ **2. Someone must assist the patient to groom self.**
- ○ **3. Patient depends entirely upon someone else for grooming needs**

(M1810) Ability to Dress Upper Body:

*Current **Ability to Dress Upper Body** safely (with or without dressing aids) including undergarments, pullovers, front-opening shirts and blouses, managing zippers, buttons, and snaps:*

- ○ **0. Able to get clothes out of closets and drawers, put them on and remove them from the upper body without assistance.**
- ● **1. Able to dress upper body without assistance if clothing is laid out or handed to the patient.**
- ○ **2. Someone must help the patient put on upper body clothing.**
- ○ **3. Patient depends entirely upon another person to dress the upper body.**

(M1820) Ability to Dress Lower Body:

*Current **Ability to Dress Lower Body** safely (with or without dressing aids) including undergarments, slacks, socks or nylons, shoes:*

- ○ **0. Able to obtain, put on, and remove clothing and shoes without assistance.**
- ● **1. Able to dress lower body without assistance if clothing and shoes are laid out or handed to the patient.**
- ○ **2. Someone must help the patient put on undergarments, slacks, socks or nylons, and shoes.**

- ◯ 3. Patient depends entirely upon another person to dress lower body.

(M1830) Bathing:

*Current ability to wash entire body safely. **Excludes grooming (washing face, washing hands, and shampooing hair).***

- ◯ 0. Able to bathe self in shower or tub independently, including getting in and out of tub/shower.
- ◯ 1. With the use of devices, is able to bathe self in shower or tub independently, including getting in and out of the tub/shower.
- ● 2. Able to bathe in shower or tub with the intermittent assistance of another person: (a) for intermittent supervision or encouragement or reminders, OR (b) to get in and out of the shower or tub, OR (c) for washing difficult to reach areas.
- ◯ 3. Able to participate in bathing self in shower or tub, but requires presence of another person throughout the bath for assistance or supervision.
- ◯ 4. Unable to use the shower or tub, but able to bathe self independently with or without the use of devices at the sink, in chair, or on commode.
- ◯ 5. Unable to use the shower or tub, but able to participate in bathing self in bed, at the sink, in bedside chair, or on commode, with the assistance or supervision of another person
- ◯ 6. Unable to participate effectively in bathing and is bathed totally by another person.

(M1840) Toilet Transferring:

Current ability to get to and from the toilet or bedside commode safely and transfer on and off toilet/commode

- ◯ 0. Able to get to and from the toilet and transfer independently with or without a device
- ● 1. When reminded, assisted, or supervised by another person, able to get to and from the toilet and transfer.
- ◯ 2. Unable to get to and from the toilet but is able to use a bedside commode (with or without assistance).
- ◯ 3. Unable to get to and from the toilet or bedside commode but is able to use a bedpan/urinal independently
- ◯ 4. Is totally dependent in toileting.

(M1845) Toileting Hygiene:

Current ability to maintain perineal hygiene safely, adjust clothes and/or incontinence pads before and after using toilet, commode, bedpan, urinal. If managing ostomy, includes cleaning area around stoma, but not managing equipment.

- ◯ 0. Able to manage toileting hygiene and clothing management without assistance.
- ● 1. Able to manage toileting hygiene and clothing management without assistance if supplies/implements are laid out for the patient.
- ◯ 2. Someone must help the patient to maintain toileting hygiene and/or adjust clothing.
- ◯ 3. Patient depends entirely upon another person to maintain toileting hygiene

(M1850) Transferring:

Current ability to move safely from bed to chair, or ability to turn and position self in bed if patient is bedfast

- ◯ 0. Able to independently transfer.
- ● 1. Able to transfer with minimal human assistance or with use of an assistive device.
- ◯ 2. Able to bear weight and pivot during the transfer process but unable to transfer self.
- ◯ 3. Unable to transfer self and is unable to bear weight or pivot when transferred by another person
- ◯ 4. Bedfast, unable to transfer but is able to turn and position self in bed.
- ◯ 5. Bedfast, unable to transfer and is unable to turn and position self

(M1860) Ambulation/Locomotion:

Current ability to walk safely, once in a standing position, or use a wheelchair, once in a seated position, on a variety of surfaces.

- ◯ 0. Able to independently walk on even and uneven surfaces and negotiate stairs with or without railings (specifically: needs no human assistance or assistive device).
- ● 1. With the use of a one-handed device (for example, cane, single crutch, hemi-walker), able to independently walk on even and uneven surfaces and negotiate stairs with or without railings.
- ◯ 2. Requires use of a two-handed device (for example, walker or crutches) to walk alone on a level surface and/or requires human supervision or assistance to negotiate stairs or steps or uneven surfaces.
- ◯ 3. Able to walk only with the supervision or assistance of another person at all times.
- ◯ 4. Chairfast, unable to ambulate but is able to wheel self independently.
- ◯ 5. Chairfast, unable to ambulate and is unable to wheel self
- ◯ 6. Bedfast, unable to ambulate or be up in a chair.

FUNCTIONAL ASSESSMENT (MOBILITY & MUSCULOSKELETAL)

(GG0170) Mobility

(GG0170A) A. Roll left and right:

The ability to roll from lying on back to left and right side, and return to lying on back on the bed.

- ◯ 06. Independent
- ● 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170B) B. Sit to lying:

The ability to move from sitting on side of bed to lying flat on the bed.

- ◯ 06. Independent
- ● 05. Setup or clean-up assistance
- ◯ 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170C) C. Lying to sitting on side of bed:

The ability to move from lying on the back to sitting on the side of the bed with feet flat on the floor, and with no back support.

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ● 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170D) D. Sit to stand:

The ability to come to a standing position from sitting in a chair, wheelchair, or on the side of the bed.

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ● 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 01. Dependent
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170E) E. Chair/bed-to-chair transfer:

The ability to transfer to and from a bed to a chair (or wheelchair).

- ◯ 06. Independent
- ◯ 05. Setup or clean-up assistance
- ● 04. Supervision or touching assistance
- ◯ 03. Partial/moderate assistance
- ◯ 02. Substantial/maximal assistance
- ◯ 07. Patient refused
- ◯ 09. Not applicable
- ◯ 10. Not attempted due to environmental limitations
- ◯ 88. Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

- ○ 01. Dependent

(GG0170F) F. Toilet transfer:

The ability to get on and off a toilet or commode.

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ● 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170G) G. Car transfer:

The ability to transfer in and out of a car or van on the passenger side. Does not include the ability to open/close door or fasten seat belt.

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ● 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170I) I. Walk 10 feet:

Once standing, the ability to walk at least 10 feet in a room, corridor, or similar space. If SOC/ROC performance is coded 07, 09, 10 or 88, skip to GG0170M, 1 step (curb)

- ○ 06. Independent
- ● 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170J) J. Walk 50 feet with two turns:

Once standing, the ability to walk 50 feet and make two turns.

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ● 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170K) K. Walk 150 feet:

Once standing, the ability to walk at least 150 feet in a corridor or similar space.

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ● 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170L) L. Walking 10 feet on uneven surfaces:

The ability to walk 10 feet on uneven or sloping surfaces (indoor or outdoor), such as turf or gravel.

- ○ 06. Independent
- ● 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170M) M. 1 step (curb):

The ability to go up and down a curb and/or up and down one step. If SOC/ROC performance is coded 07, 09, 10 or 88, skip to GG0170P, Picking up object.

- ○ 06. Independent
- ● 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170N) N. 4 steps:

The ability to go up and down four steps with or without a rail. If SOC/ROC performance is coded 07, 09, 10 or 88, skip to GG0170P, Picking up object.

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ● 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170O) O. 12 steps:

The ability to go up and down 12 steps with or without a rail.

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ● 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170P) P. Picking up object:

The ability to bend/stoop from a standing position to pick up a small object, such as a spoon, from the floor.

- ○ 06. Independent
- ○ 05. Setup or clean-up assistance
- ● 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170Q) Q. Does patient use a wheelchair and/or scooter?

○ 0. No ● 1. Yes ○ (-) Not assessed/no information

(GG0170R) R. Wheel 50 feet with two turns:

Once seated in wheelchair/scooter, the ability to wheel at least 50 feet and make two turns.

- ○ 06. Independent
- ☑ 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170RR) RR1. Indicate the type of wheelchair or scooter used.

○ 1. Manual ☑ 2. Motorized ○ (-) Not assessed/no information

(GG0170S) S. Wheel 150 feet:

Once seated in wheelchair/scooter, the ability to wheel at least 150 feet in a corridor or similar space.

- ○ 06. Independent
- ☑ 05. Setup or clean-up assistance
- ○ 04. Supervision or touching assistance
- ○ 03. Partial/moderate assistance
- ○ 02. Substantial/maximal assistance
- ○ 01. Dependent
- ○ 07. Patient refused
- ○ 09. Not applicable
- ○ 10. Not attempted due to environmental limitations
- ○ 88. Not attempted due to medical conditions or safety concerns
- ○ (-) Not assessed/no information

(GG0170SS) SS1. Indicate the type of wheelchair or scooter used.

○ 1. Manual ☑ 2. Motorized ○ (-) Not assessed/no information

Gait and Mobility Abnormalities:

- ☐ No Problems Identified
- ☑ Weakness
- ☑ Poor Balance
- ☐ Bed Bound
- ☐ Chair Bound
- ☐ Joint Stiffness
- ☐ Shuffling/Wide-based Gait
- ☐ Orthopedic Aftercare
- ☑ High Risk for Falls
- ☐ Weight Bearing Restriction
- ☐ Poor Conditioning
- ☐ Fracture:
- ☐ Joints Pain:
- ☐ Amputation:
- ☐ Atrophy:
- ☐ Paresthesia:
- ☑ Contractures (Site): right arm and hand
- ☐ Foot Drop: ☐ Right ☐ Left

Fall(s) since last visit (Specify):

Weakness (Specify):

Change in ADL (Explain):

Comments:

Range of Motion:

☑ Limited Range of Motion: ☑ RUE ☑ RLE ☐ LUE ☐ LLE ☐ Quadriplegia ☑ Hemiplegia ☐ Paraplegia

Comments:

Grip Strength:

☑ Normal ☐ Strong ☐ Weak ☐ Equal ☑ Unequal ☐ Bilateral ☑ Right ☐ Left

Others (Specify):

Comments:

DME and Supplies (POC Element (§484.60 (2.iii))):

DME Lookup

Hemicane, Motorized Wheelchair

#18B Activities Permitted (POC Element (§484.60 (2.viii))):

- ○ 1- Complete bedrest
- ○ 2 - Bedrest/BRP
- ☑ 3 - Up as tolerated
- ○ 4 - Transfer bed/chair
- ☑ 5 - Exercises prescribed
- ○ 6 - Partial weight bearing
- ○ 7 - Independent in home
- ○ 8 - Crutches
- ☑ 9 - Cane
- ☑ A - Wheelchair
- ○ B - Walker
- ○ C - No restrictions
- ○ D - Other (specify)

Rehabilitation Potential (§484.60 (2.vi))):

- ○ Poor
- ☑ Fair
- ○ Good
- ○ Excellent

Others

ENDOCRINE (DIABETIC MANAGEMENT)

Endocrine

Is patient diabetic? ☐ Yes ☑ No **If Yes, How Long?**

Diabetes Type: ☐ Insulin-Dependent Diabetes Mellitus (IDD/Type 1) ☐ Non-Insulin-Dependent Diabetes Mellitus (NIDDM/Type 2)

Insulin Administered By: ☐ Patient ☐ Caregivier ☐ SN

Glucose Level Issue: ☐ Hypoglycemia (Low Blood Sugar) ☐ Hyperglycemia (High Blood Sugar)

Frequency of Blood Sugar Issues: ☐ Daily ☐ Weekly ☐ Monthly ☐ Rarely ☐ As needed

Does patient or caregiver routinely perform inspection of the patient's lower extremities? ☐ Yes ☐ No ☐ N/A

Symptoms & Observations:

- ☐ Fatigue (related to blood sugar)
- ☐ Shakiness/Tremors
- ☐ Sweating
- ☐ Confusion/Irritability
- ☐ Hunger
- ☐ Dizziness
- ☐ Blurred Vision
- ☐ Dry Mouth/Excessive Thirst

☐ Frequent Urination ☐ Weight Loss (unintentional) ☐ Weight Gain (unintentional)

Specify Other Symptoms or Patterns:

PLAN OF CARE ORDERS AND DISCHARGE PLAN

Plan of Care Orders

(POC Element):

Orders and Treatment (Specify Frequency, Duration and Purpose of Visits): §484.60 (2.iv)

SN Frequency: 2/wk x 1wk, 1/wk x 3wks **MSW Frequency:** **PT Frequency: Evaluation and Treatment** **OT Frequency: Evaluation and Treatment**

HHA Frequency: **ST Frequency** **Dietitian Frequency:**

MD SIGNATURE COVERS ALL PAGES

ADMIT PATIENT TO ACTIVE HOME HEALTH SERVICES. ALL DIAGNOSIS AND MEDICATIONS VERIFIED WITH THE PRIMARY CARE PHYSICIAN.

Robert Bryant, a 71-year-old male patient admitted to home health care because they have significant functional impairments and medical conditions that severely limit their ability to leave the home without assistance. The patient is considered homebound as the patient ambulates with a hemi-cane and uses a motorized wheelchair for long distances due to right-sided hemiparesis following a cerebrovascular accident (CVA), which causes generalized muscle weakness, poor endurance, unsteady gait, and poor balance. The patient fatigues easily and experiences dyspnea with moderate exertion, making any attempt to leave the home a considerable and taxing effort. Additionally, the patient requires assistance with all activities of daily living, including grooming, bathing, dressing, transferring, toileting, and managing medications.

The patient's home environment also presents safety concerns, such as the presence of rugs in hallways, lack of grab bars in the bathroom, and poor lighting, which increase the risk of falls and injury if the patient attempts to leave the home independently. The patient has a history of urinary incontinence and contractures affecting the right arm and hand, further contributing to functional limitations. Allowable absences from the home are limited to medically necessary appointments, as outpatient therapy is currently deferred pending evaluation of palpitations related to medication.

Given these limitations and safety concerns, skilled nursing services are medically necessary to monitor vital signs, provide education on disease processes and medication management, assess for progression of disease, and reduce the risk of hospitalization and injury. Occupational therapy is also indicated to improve upper body strength, conditioning, and functional status to maximize the patient's independence and safety at home. These documented functional deficits and medical conditions clearly meet Medicare criteria for homebound status and justify the need for skilled home health services.

Frequency and Duration:
HH SN frequency: 2 times a week for 1 week, 1 time a week for 3 weeks for skilled observation and assessment, medication reconciliation, pain assessment, wound monitoring and management, infection control measures, diet, home safety and disease process education.
HH PT frequency: For evaluation and treatment for mobility, gait, transfer, establish therapeutic exercises, strengthening, home safety and fall prevention techniques training effective on week of 04/16/2026.
HH OT frequency: For evaluation and treatment focused on improving activities of daily living (ADLs), fine and gross motor skills, cognitive-perceptual function, adaptive techniques, and caregiver education.

Goals:
1. Patient will perform grooming, bathing, and dressing with only setup or supervision (no hands-on assistance) for at least 5 of 7 days within 4 weeks.
2. Patient will participate in a toileting schedule and pelvic floor/behavioral strategies with caregiver assistance to reduce urinary incontinence episodes by 50% from baseline within 4 weeks.
3. Patient will report average right shoulder pain ≤3/10 at rest and ≤5/10 with activity and prescribed non-pharmacologic measures (ice/positioning), with documented decrease in pain episodes on 5 of 7 days within 4 weeks.
4. Patient will adhere to prescribed bowel regimen and report a spontaneous bowel movement at least every 48–72 hours, with reduced reports of constipation within 4 weeks.
5. Patient or caregiver will, implement recommended home safety modifications (installation/use of grab bars, removal of loose rugs, improvement of lighting) and demonstrate safe navigation of primary living areas within 4 weeks.

Conference With:

☑ MD ☑ SN ☑ PT ☑ OT ☐ ST ☐ MSW ☐ HHA ☐ DON/DOPCS ☑ Clinical Coordinator ☑ Clinical Manager

Name:
Aimee Floresca, RN, CM

Regarding:
SOC, POC, patient's status and frequency

Physician Contacted Re:
SOC, POC, patient's status and frequency

Order Changes:
none

Plan for Next Visit:
Will continue to assess and evaluate patient understanding, functional status, and knowledge of self-care management on the next visit.

Next Physician Visit:
mm/dd/yyyy

Patient and caregiver education and training to facilitate timely discharge (POC Element (§484.60 (2.xiii))):

Discharge Plans

(POC Element):

When patient knows when to notify physician.
Discharge when reliable caregiver available to assist with patient's medical needs.
Discharge when patient is independent in management of medical needs.
When goals are met.
Able to return to an independent level of care.
Able to understand medication regime and care related to diagnoses.
Discharge to caregiver.
Discharge when medical condition is stable and patient is no longer in need of skilled services.
Able to stay in residence with primary caregiver's assistance or community agency's support.
Discharge to care of physician.
Discharge when caregiver willing and able to manage all aspects of patient's care.
Discharge patient to self-care.

☐ **Written notice of discharge provided to patient**

Discharge scheduled for:
mm/dd/yyyy

Discharge Plan Discussed with:

☑ Patient/Family ☐ Care Manager ☑ Physician ☐ Other (specify):

Discharge Plan:

☑ To self-care when goals met ☑ To self care when max potential achieved ☐ To outpatient therapy with MD approval

☐ Others (Specify):

Comments:

PATIENT SUMMARY & CLINICAL NARRATIVE

Summary

(POC Element):

CARE PLAN (PROBLEMS / GOALS / INTERVENTIONS)

Patient Care Plan / Goals / Interventions:

Problem: Vital Signs Assessment
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Patient's vital signs will be within normal limits.
Interventions: Assigned Staff: SN
Check O2 saturation every visit for any shortness of breath on exertion, fatigue/weakness.
Progress Toward Goals:
No Progress Yet
Check VS every visit and report to PMD if patient vitals are not within the parameters.
Progress Toward Goals:
No Progress Yet

Problem: Cardiopulmonary
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Demonstrate pursed-lip breathing exercises, deep breathing exercises, and energy-conserving measures.
Interventions: Assigned Staff: SN
Review patient/caregiver on methods to conserve energy, frequent rest periods, sitting instead of standing to perform tasks, and performing slower, purposeful movements which will limit fatigue.
Progress Toward Goals:
No Progress Yet
Instruct patient/caregiver on pursed lip breathing exercises, deep breathing exercises to alleviate dyspnea.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Patient will be free from signs/symptoms of cardiac and respiratory distress during the certification period. Verbalize understanding of symptoms of cardiac complications and when to call 911.
Interventions: Assigned Staff: SN
Reinforce patient/caregiver on sodium limitations, managing stress, avoiding smoking/alcohol intake, and compliance with prescribed medications to control blood pressure.
Progress Toward Goals:
No Progress Yet
Encourage the patient/caregiver to provide a quiet environment, limiting stimuli to promote rest.
Progress Toward Goals:
No Progress Yet
Advise the patient to get up slowly to offset the feeling of dizziness.
Progress Toward Goals:
No Progress Yet
Instruct patient/caregiver to notify SN/MD for signs/symptoms of cardiac and respiratory distress like chest pain, cyanosis, dependent edema, jugular vein distention, palpitations, wheezing, increased respiratory rate, anxiety, headache, lightheadedness, and shortness of breath.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Patient will be free from cardiopulmonary distress during the care episode. Patient's pulse and blood pressure will be within established parameters.
Interventions: Assigned Staff: SN
Reinforce patient/caregiver to check and record blood pressure, including heart rate.
Progress Toward Goals:
No Progress Yet
Assess cardiopulmonary status every visit.
Progress Toward Goals:
No Progress Yet

Problem: Hemiplegia following cerebral infarct affecting right dominant side
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Improve mobility and strength on the affected side
Interventions: Assigned Staff: SN
Provide education to family on safe transfer techniques to assist with mobility.
Progress Toward Goals:
No Progress Yet
Assist with the use of adaptive equipment to improve mobility.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Enhance activities of daily living (ADLs) independence
Interventions: Assigned Staff: SN
Assess the home environment for safety and make recommendations for modifications.
Progress Toward Goals:
No Progress Yet
Provide ongoing evaluation of the patient's ability to perform ADLs and adjust the care plan accordingly.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Promote nutritional health and hydration
Interventions: Assigned Staff: SN
Monitor the patient's weight and hydration status regularly.
Progress Toward Goals:
No Progress Yet
Provide strategies for enhancing appetite and ensuring adequate fluid intake.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Address psychosocial needs and emotional well-being
Interventions: Assigned Staff: SN
Encourage participation in social activities to reduce isolation.
Progress Toward Goals:
No Progress Yet
Educate family members on the importance of emotional support and active listening.
Progress Toward Goals:
No Progress Yet

Problem: Essential (primary) hypertension
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing

Maintain blood pressure within the target range of 120/80 mmHg.
Interventions: Assigned Staff: SN
Monitor blood pressure daily and keep a log of readings to identify patterns.
Progress Toward Goals:
No Progress Yet
Educate the patient and family on the importance of adherence to prescribed antihypertensive medications.
Progress Toward Goals:
No Progress Yet
Schedule regular follow-up appointments to assess blood pressure and medication effectiveness.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Educate the patient and family about hypertension and its complications.
Interventions: Assigned Staff: SN
Conduct educational sessions with the patient and family regarding hypertension management.
Progress Toward Goals:
No Progress Yet
Provide written materials on the effects of hypertension and the importance of treatment.
Progress Toward Goals:
No Progress Yet
Utilize teach-back methods to ensure understanding of hypertension management.
Progress Toward Goals:
No Progress Yet

Problem: Polyneuropathy, unspecified
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Improve patient's mobility and reduce risk of falls.
Interventions: Assigned Staff: SN
Conduct a comprehensive mobility assessment to identify risk factors and develop a tailored exercise program.
Progress Toward Goals:
No Progress Yet
Implement a balance training program to enhance stability and coordination.
Progress Toward Goals:
No Progress Yet
Educate the patient and family on home safety modifications to prevent falls.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Enhance patient's ability to perform daily living activities.
Interventions: Assigned Staff: SN
Develop a personalized plan for adaptive techniques to assist with ADLs.
Progress Toward Goals:
No Progress Yet
Provide training for the patient and family on the use of adaptive equipment.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Manage pain and discomfort associated with polyneuropathy.
Interventions: Assigned Staff: SN
Conduct regular assessments of pain levels and their impact on daily functioning.
Progress Toward Goals:
No Progress Yet
Educate the patient and family on pain management techniques, including medication management and relaxation strategies.
Progress Toward Goals:
No Progress Yet
Implement a medication management plan in collaboration with the prescribing physician.
Progress Toward Goals:
No Progress Yet

Problem: Other specified disorders of the male genital organs
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Manage pain and discomfort associated with the disorder.
Interventions: Assigned Staff: SN
Provide medication management and pain assessment.
Progress Toward Goals:
No Progress Yet
Educate the patient and family about pain management techniques.
Progress Toward Goals:
No Progress Yet
Monitor vital signs and report any changes indicative of increased pain.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Enhance the patient's understanding of their condition and treatment options.
Interventions: Assigned Staff: SN
Provide educational materials about the disorder and treatment plans.
Progress Toward Goals:
No Progress Yet
Assess the patient's learning needs and adjust the education plan accordingly.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Improve the patient's emotional well-being and coping strategies.
Interventions: Assigned Staff: SN
Provide emotional support and reassurance during visits.
Progress Toward Goals:
No Progress Yet
Assess the patient's mental health status and refer to mental health services if needed.
Progress Toward Goals:
No Progress Yet
Implement coping strategies and stress relief techniques.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Promote overall health and prevent complications.
Interventions: Assigned Staff: SN
Conduct regular assessments of the patient's physical health.
Progress Toward Goals:
No Progress Yet
Monitor for signs of infection or complications related to the disorder.
Progress Toward Goals:
No Progress Yet

Encourage a healthy lifestyle, including diet and exercise recommendations.
Progress Toward Goals:
No Progress Yet

Problem: Constipation, unspecified
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Achieve regular bowel movements at least 3 times a week.
Interventions: Assigned Staff: SN
Monitor fluid intake to ensure adequate hydration.
Progress Toward Goals:
No Progress Yet
Assess the patient's medication for potential side effects that may contribute to constipation.
Progress Toward Goals:
No Progress Yet
Encourage light physical activity as tolerated to stimulate bowel function.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Reduce the patient's discomfort related to constipation.
Interventions: Assigned Staff: SN
Assist the patient with personal hygiene to maintain comfort.
Progress Toward Goals:
No Progress Yet
Monitor the patient's abdominal distension and report changes to the healthcare team.
Progress Toward Goals:
No Progress Yet
Encourage the family to assist in monitoring the patient's bowel habits.
Progress Toward Goals:
No Progress Yet

Problem: Other visual disturbances
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Improve patient's ability to perform daily activities safely.
Interventions: Assigned Staff: SN
Provide training on using assistive devices for better visibility.
Progress Toward Goals:
No Progress Yet
Educate family members on how to assist the patient with safe mobility.
Progress Toward Goals:
No Progress Yet
Monitor visual acuity and report changes to the healthcare provider.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Enhance communication regarding the patient's visual disturbances with family and caregivers.
Interventions: Assigned Staff: SN
Encourage family participation in therapy sessions to understand the challenges faced by the patient.
Progress Toward Goals:
No Progress Yet
Establish regular check-ins with family members to discuss progress and concerns.
Progress Toward Goals:
No Progress Yet
Document and share observations of visual disturbances to ensure continuity of care.
Progress Toward Goals:
No Progress Yet

Problem: Fall Risk
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Reduce the likelihood of falls in the home environment.
Interventions: Assigned Staff: SN
Educate family members on fall prevention strategies.
Progress Toward Goals:
No Progress Yet
Ensure proper medication review and management to minimize side effects that may contribute to falls.
Progress Toward Goals:
No Progress Yet
Install assistive devices (e.g., grab bars, non-slip mats) in key areas of the home.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Enhance the patient’s mobility and strength.
Interventions: Assigned Staff: SN
Provide training on the use of mobility aids (e.g., walkers, canes).
Progress Toward Goals:
No Progress Yet
Encourage family involvement in physical activities to promote strength and mobility.
Progress Toward Goals:
No Progress Yet
Monitor and track the patient’s progress with mobility goals.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Educate patient and family on recognizing signs of dizziness or instability.
Interventions: Assigned Staff: SN
Teach the patient and family about the importance of hydration and nutrition in preventing falls.
Progress Toward Goals:
No Progress Yet
Implement a daily check-in routine for family members to assess the patient's condition.
Progress Toward Goals:
No Progress Yet
Encourage the use of a fall alert system for immediate response.
Progress Toward Goals:
No Progress Yet

Problem: Pressure Ulcer Prevention
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Verbalize understanding of the importance of pressure-relieving and proper incontinence care measures.
Interventions: Assigned Staff: SN
Instruct caregiver to change patient’s pads/underwear frequently or as needed when heavily soiled to prevent skin breakdown related to excessive moisture from urine/stool.
Progress Toward Goals:
No Progress Yet
Instruct patient/caregiver to cleanse skin with mild soap and water then pat dry thoroughly after every incontinence. Instruct patient to wipe perineum from front to back to prevent infection.
Progress Toward Goals:

No Progress Yet
Instruct patient/caregiver on pressure-relieving measures such as repositioning every 2 hours, float heels, pad bony prominences and methods to reduce friction and shear.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
The patient's skin remains intact.
Interventions: Assigned Staff: SN
Instruct patient/caregiver to keep skin clean and dry to prevent skin breakdown.
Progress Toward Goals:
No Progress Yet
Instruct patient/caregiver to inspect skin daily and notify SN/MD for redness or open sores.
Progress Toward Goals:
No Progress Yet
Assess skin for breakdown due to risk of pressure ulcer development.
Progress Toward Goals:
No Progress Yet

Problem: Rehospitalization Risk and Medication Management
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Patient will be free from side effects and adverse medication reactions during the entire episode. Patient/Caregiver will demonstrate medication safety and compliance. Patients will have no re-hospitalizations.
Interventions: Assigned Staff: SN
Identify Medication Interactions and evaluate patient/family/caregiver(s) understanding of medication compliance.
Progress Toward Goals:
No Progress Yet

Ongoing Care Plans within OASIS date (04/16/2026):

Assigned Staff: Dizon, Bright RN